

Tablets

Composition

Each tablet contains Metoclopramide Hydrochloride equivalent to Anhydrous Metoclopramide Hydrochloride 10mg.

Pharmacodynamics

The action of metoclopramide is closely associated with parasympathetic nervous control of the upper gastro-intestinal tract where it has the effect of encouraging normal peristaltic action. This provides for a fundamental approach to the control of those conditions, where disturbed gastrointestinal motility is a common underlying factor.

Pharmacokinetics

Metoclopramide is metabolised in the liver and the predominant route of elimination of metoclopramide and its metabolites is via the kidney.

Renal impairment

The clearance of metoclopramide is reduced by up to 70% in patients with severe renal impairment, while the plasma elimination half-life is increased (approximately 10 hours for a creatinine clearance of 10-50 mL/minute and 15 hours for a creatinine clearance <10 mL/minute).

Hepatic impairment

In patients with cirrhosis of the liver, accumulation of metoclopramide has been observed, associated with a 50% reduction in plasma clearance.

Indication

Adults

Emeliv is indicated in adults for:

- Prevention of delayed chemotherapy induces nausea and vomiting (CINV)
- Prevention of radiotherapy induced nausea and vomiting (RINV)
- Symptomatic treatment of nausea and vomiting, including acute migraine induced nausea and vomiting

Paediatric

Emeliv is indicated in children (aged 1-18 years) for:

- Prevention of delayed chemotherapy induces nausea and vomiting (CINV) as a second line option

Recommended Dosage

To be administered orally.

Adults

The recommended single dose is 10mg, repeated up to three times daily.

The maximum recommended daily dose is 30 mg or 0.5mg/kg body weight.

The maximum recommended treatment duration is 5 days.

Prevention of delayed chemotherapy induced nausea and vomiting (CINV) (paediatric patients aged 1-18 years)

The recommended dose is 0.1 to 0.15 mg/kg body weight, repeated up to three times daily by oral route. The maximum dose in 24 hours is 0.5 mg/kg body weight.

Dosing Table

Age	Body Weight	Dose	Frequency
1-3 years	10-14 kg	1mg	Up to 3 times daily
3-5 years	15-19 kg	2mg	Up to 3 times daily
5-9 years	20-29 kg	2.5mg	Up to 3 times daily
9-18 years	30-60 kg	5mg	Up to 3 times daily
15-18 years	Over 60kg	10mg	Up to 3 times daily

The maximum treatment duration is 5 days for prevention of delayed chemotherapy induced nausea and vomiting (CINV). Tablets are not suitable for use in children weighing less than 30kg. Other pharmaceutical forms may be more appropriate for administration to this population.

Frequency of administration

A minimum interval of 6 hours between two administrations is to be respected, even if vomiting or rejection of the dose occurs.

Special population

Elderly

In elderly patients a dose reduction should be considered, based on renal and hepatic function and overall frailty.

Renal impairment

In patients with end stage renal disease (Creatinine clearance ≤ 15 ml/min), the daily dose should be reduced by 75%.

In patients with moderate to severe renal impairment (Creatinine clearance 15-60 ml/min), the dose should be reduced by 50%.

Hepatic impairment

In patients with severe hepatic impairment, the dose should be reduced by 50%.

Contraindications

This medicine is contraindicated for:

- Hypersensitivity to the active substance or to any of the excipients listed
- Gastrointestinal haemorrhage, mechanical obstruction or gastro-intestinal perforation for which the stimulation of gastrointestinal motility constitutes a risk
- Confirmed or suspected pheochromocytoma, due to the risk of severe hypertension episodes
- History of neuroleptic or metoclopramide-induced tardive dyskinesia
- Epilepsy (increased crises frequency and intensity)
- Parkinson's disease
- Combination with levodopa or dopaminergic agonists
- Known history of methaemoglobinaemia with metoclopramide or of NADH cytochrome-b5 deficiency
- Use in children less than 1 year of age

Warning and Precautions

- Avoid doses exceeding 0.5mg/kg/day
- Extrapyramidal effects, especially dystonic reaction of metoclopramide are more likely to occur in children shortly after initiation of therapy, and usually with doses higher than 0.5mg per kg of body weight per day.

Neurological disorders

Extrapyramidal disorders may occur, particularly in children and young adults, and/or when high doses are used. These reactions generally occur at the beginning of treatment, and can occur after a single dose. If extrapyramidal symptoms occur, metoclopramide should be discontinued immediately. These effects are generally completely reversible after treatment discontinuation; however, symptomatic treatment may be required (benzodiazepines in children, and/or anticholinergic antiparkinsonian medicinal products in adults).

An interval of at least six hours should be respected between each dose even if vomiting or rejection of the dose occurs, in order to avoid overdose.

Long-term treatment with metoclopramide may cause potentially irreversible tardive dyskinesia, particularly in elderly subjects. Treatment should not exceed 3 months because of the risk of tardive dyskinesia. Treatment must be discontinued if clinical signs of tardive dyskinesia occur.

Neuroleptic malignant syndrome has been described with metoclopramide in combination with neuroleptics and with metoclopramide monotherapy. Metoclopramide must be immediately discontinued if symptoms of neuroleptic malignant syndrome develop, and appropriate treatment should be initiated.

Particular caution should be exercised in patients with underlying neurological disorders, and in patients receiving other centrally-acting drugs.

Symptoms of Parkinson's disease may also be exacerbated by metoclopramide.

Methemoglobinemia

Methemoglobinemia, which could be related to NADH-cytochrome b5 reductase deficiency, has been reported. If this occurs, treatment must be immediately and permanently discontinued, and appropriate measures initiated (such as treatment with methylene blue).

Cardiac disorders

Serious cardiovascular undesirable effects, including cases of severe bradycardia, circulatory collapse, cardiac arrest and QT prolongation have been reported during administration of metoclopramide by injection, particularly via the intravenous route.

Particular caution should be exercised when administering metoclopramide, particularly via the intravenous route, in elderly subjects, patients with cardiac conduction disorders (including QT prolongation), patients with electrolyte imbalance, bradycardia, and patients taking other drugs known to prolong QT interval.

The intravenous injection must be given as a slow bolus (of at least 3 minutes' duration) in order to reduce the risk of undesirable effects (e.g. hypotension, akathisia).

Kidney or liver failure

In patients with kidney failure or severe liver failure, a dose reduction is recommended.

Metoclopramide may cause elevation of serum prolactin levels.

Patients with rare hereditary problems of galactose intolerance, the Lapp lactose deficiency of glucose-galactose malabsorption should not take this medicine.

Care should be exercised when using *Emeliv* in patients with a history of atopy (including asthma) or porphyria.

Metoclopramide should not be used in the immediate post-operative period (up to 3-4 days) following pyloroplasty or gut anastomosis, as vigorous gastrointestinal contractions may adversely affect healing.

Interactions with Other Medicines

Contraindicated combination

Levodopa or dopaminergic agonists and metoclopramide have a mutual antagonism.

Combination to be avoided

Alcohol potentiates the sedative effect of metoclopramide.

Combination to be taken into account

Due to the prokinetic effect of metoclopramide, the absorption of certain drugs may be modified.

Anticholinergics and morphine derivatives

Anticholinergics and morphine derivatives may have both a mutual antagonism with metoclopramide on the digestive tract motility.

Central nervous system depressants

(Morphine derivatives, anxiolytics, sedative H1 antihistamines, sedative antidepressants, barbiturates, clonidine and related)

Sedative effects of Central Nervous System depressants and metoclopramide are potentiated.

Neuroleptics

Metoclopramide may have an additive effect with other neuroleptics on the occurrence of extrapyramidal disorders.

Serotonergic drugs

The use of metoclopramide with serotonergic drugs such as SSRIs may increase the risk of serotonin syndrome.

Digoxin

Metoclopramide may decrease digoxin bioavailability. Careful monitoring of digoxin plasma concentration is required.

Cyclosporine

Metoclopramide increases cyclosporine bioavailability. Careful monitoring of cyclosporine plasma concentration is required. The clinical consequence is uncertain.

Strong CYP2D6 inhibitors

Metoclopramide exposure levels are increased when co-administered with strong CYP2D6 inhibitors such as fluoxetine and paroxetine. Although the clinical significance is uncertain, patients should be monitored for adverse reactions.

Emeliv may reduce plasma concentrations of atovaquone.

Pregnancy & Lactation

Pregnancy

Metoclopramide should be avoided at the end of pregnancy. If metoclopramide is used, neonatal monitoring should be undertaken.

Breast-feeding

Metoclopramide is excreted in breast milk at low level. Adverse reactions in the breast-fed baby cannot be excluded. Therefore metoclopramide is not recommended during breastfeeding. Discontinuation of metoclopramide in breastfeeding women should be considered.

Women who are pregnant or breast feeding infants should therefore be advised to consult their doctor for appropriate treatment.

Effects on ability to drive and use machines

Emeliv has moderate influence on the ability to drive and use machines. Metoclopramide may cause drowsiness, dizziness, dyskinesia and dystonias which could affect the vision and also interfere with the ability to drive and operate machinery.

Side Effects

Table 1: Adverse Drug Reactions

System Organ Class	Frequency	Adverse Reactions
Blood and lymphatic system disorders	Not known	Methaemoglobinaemia, which could be related to NADH cytochrome b5 reductase deficiency, particularly in neonates; Sulfhaemoglobinaemia, mainly with concomitant administration of high doses of sulphur-releasing medicinal products
Cardiac disorders	Uncommon	Bradycardia, particularly with intravenous formulation
	Not known	Cardiac arrest, occurring shortly after injectable use, and which can be subsequent to bradycardia; Atrioventricular block, Sinus arrest particularly with intravenous formulation; Electrocardiogram QT prolonged; Torsade de Pointes
Endocrine disorders*	Uncommon	Amenorrhoea, Hyperprolactinaemia
	Rare	Galactorrhoea
	Not known	Gynaecomastia
Gastrointestinal Disorders	Common	Diarrhoea
General Disorders and Administration Site Conditions	Common	Asthenia
Immune system disorders	Uncommon	Hypersensitivity
	Not known	Anaphylactic reaction (including anaphylactic shock particularly with intravenous formulation)
Nervous system disorders	Very common	Somnolence
	Common	Extrapyramidal disorders (particularly in children and young adults and/or when the recommended dose is exceeded, even following administration of a single dose of the drug), Parkinsonism, Akathisia
	Uncommon	Dystonia, Dyskinesia, Depressed level of consciousness
	Rare	Convulsion especially in epileptic patients
	Not known	Tardive dyskinesia which may be persistent, during or after prolonged treatment, particularly in elderly patients, Neuroleptic malignant syndrome
Psychiatric disorders	Common	Depression
	Uncommon	Hallucination
	Rare	Rare Confusional state
Vascular disorder	Common	Hypotension, particularly with intravenous formulation
	Not known	Shock, syncope after injectable use, Acute hypertension in patients with phaeochromocytoma Transient increase in blood pressure

* Endocrine disorders during prolonged treatment in relation with hyperprolactinaemia (amenorrhoea, galactorrhoea, gynaecomastia).

The following reactions, sometimes associated, occur more frequently when high doses are used:

- Extrapyramidal symptoms: acute dystonia and dyskinesia, parkinsonian syndrome, akathisia, even following administration of a single dose of the medicinal product, particularly in children and young adults.
- Drowsiness, decreased level of consciousness, confusion and hallucination.

Overdose & treatment

Extrapyramidal disorders, drowsiness, decreased level of consciousness, confusion, hallucination, and cardio-respiratory arrest may occur.

In case of extrapyramidal symptoms related or not to overdose, the treatment is only symptomatic (benzodiazepines in children and/or anticholinergic anti-parkinsonian medicinal products in adults).

A symptomatic treatment and a continuous monitoring of the cardiovascular and respiratory functions should be carried out according to clinical status.

Presentation

White, round, shallow convex tablet with single breakline, 7.9 mm in diameter in blister packs of 100's.

Shelf-life

The expiry date is indicated on the packaging.

Storage

Store below 30°C. Protect from light.

KEEP OUT OF REACH OF CHILDREN

JAUHI DARI KANAK-KANAK

For further information, please consult your pharmacist or physician.

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Manufacturer and Product Registration Holder

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